A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Pamela Dagley
NIIM Wellness Clinic

Dear Dr Dagley,

Re: Ms Lynne Frean
DOB: 12/11/1952 Phone: 0431 308 472
Address: 3 Glass Street, KEW EAST, VIC, 3102
Email: lynnefrean@hotmail.com

Summary

- **Normal gastroscopy and colonoscopy with normal biopsies.**
- **Surveillance colonoscopy interval extended to five years.**
- **For abdominal ultrasound to investigate abdominal aortic pulsation.**
- **No action required by you.**

I had the pleasure of reviewing Lynne following her recent gastroscopy and colonoscopy. You will be pleased to know that the procedures did not reveal any significant pathology.

The colonoscopy was entirely clear with no evidence of malignancy, polyps, or inflammation. This is particularly reassuring given that her previous colonoscopy two years ago required the removal of a sessile serrated adenoma. Biopsies taken of the bowel were histologically normal. Regarding her gastroscopy, the oesophagus and small intestine appeared healthy, and the duodenal biopsies were normal. There was some evidence of mild inflammation in the stomach; however, the biopsies excluded both Helicobacter pylori and any features of malignancy. I also noted that a hiatus hernia, which had been seen previously, was not prominent during this examination, which likely reflects the known intermittent visibility of smaller hernias.

We briefly discussed her ongoing spinal and neurological issues. Lynne is currently managed conservatively under the guidance of a physiotherapist and has sought a second specialist opinion. She is not currently using non-steroidal anti-inflammatory drugs to manage her symptoms, which is helpful from a gastrointestinal perspective.

Impression and Plan Lynne reported a recent episode of a "pounding" sensation in her abdomen, just above the umbilicus. While this was a one-off event, I suspect it may represent the pulsation of the abdominal aorta, particularly as Lynne has a thin habitus. While the risk of an aneurysm is low, I have organised an abdominal ultrasound for her peace of mind to evaluate the abdominal aorta.

Given that the current colonoscopy was normal, I have recommended that her next surveillance colonoscopy be deferred for five years. This interval may be reconsidered in the future depending on her overall medical comorbidities at that time. I will review the results of her ultrasound once available and will only contact her or your rooms further if there are findings that require intervention.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

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